
DEPOCON INJECTION

DESCRIPTION:

Is a clear yellowish oily solution.

COMPOSITION:

Each ml contains 200 mg of Norethisterone Enanthate in oily solution.

PHARMACODYNAMICS:

Norethisterone enanthate is a long-active derivative of norethisterone which has been used similarly to medroxyprogesterone acetate as a 'progestogen only' contraceptive.

Depocon is a depot progestogen for hormonal contraception. Protection against conception is based primarily upon an alteration of the cervical mucus which is present for the whole of the duration of action and which prevents the ascent of the sperms into the uterine cavity. Radioimmunological studies have shown that, within the first 5-7 weeks after injection, ovulation is suppressed as a result of the high plasma level of norethisterone. In addition, Depocon causes morphological changes in the endometrium which can be considered as having the effect of rendering nidation of a fertilized egg difficult.

PHARMACOKINETICS:

Norethisterone is absorbed from the gastro-intestinal tract and its effects last for at least 24 hours. When injected, it is detectable in the plasma after 2 days and is not completely excreted in the urine after 5 days.

INDICATION:

For parenteral hormonal contraception Depocon is particularly suitable for women who for any reason cannot take oral contraceptives regularly or who tolerate such preparations poorly. Depocon should only be administered to women with a history of normal biphasic cycles.

RECOMMENDED DOSAGE:

Before starting Depocon, a thorough general medical and gynaecological examination (including the breasts) should be carried out and pregnancy must be excluded.

Depocon must always be injected deeply into the gluteal muscle. It is advisable to cover the site of injection afterwards with a plaster in order to prevent partial reflux of the injection solution.

The first intramuscular injection of 200 mg is given within the first five days of the cycle. The next three injections are to be given at intervals of 8 weeks, after which the injection interval should be extended to every 12 weeks (84 days). If it is extended beyond that time, contraceptive protection will be inadequate from the 13th week onwards.

If special circumstances make it necessary (e.g. holidays, journeys), the interval between injections can be shortened by 1 week.

In each case, the next injection should only be given if bleeding has occurred within the preceding 10 weeks. Depocon must otherwise be withdrawn and pregnancy ruled out.

If it appears desirable to check the full recovery of ovarian function in young women, the preparation should be discontinued after 2 years of administration.

Following discontinuation, normal ability to conceive usually returns about 4-5 months after the last injection.

If a physiological cycle pattern fails to develop within this period of time, appropriate treatment is indicated in women who want children.

As this preparation contains benzyl alcohol, its use should be avoided in children under two years of age.
Not to be used in neonates.

ROUTE OF ADMINISTRATION:

For i.m injection.

CONTRACINDICATIONS:

Depocon, like all hormonal contraceptives, is contra-indicated in pregnancy, pathological increased blood pressure, acute and severe, chronic liver diseases (particularly primary biliary liver cirrhosis), a history of herpes of pregnancy, a history of idiopathic jaundice of pregnancy or severe pruritus of pregnancy, Dubin-Johnson syndrome, Rotor syndrome. In other forms of liver function, the risk benefit ratio must be considered carefully. It is recommended as a precaution that Depocon should not be used where there is a history or existing thromboembolic processes (e.g. stroke, myocardial infarction) or in sickle-cell anaemia.

WARNINGS & PRECAUTIONS:

It has been concluded from epidemiological surveys that the use of oral ovulation inhibitors (combined preparations containing estrogen) can lead to an increased "relative risk" of thromboembolic diseases. Although no such observations have so far been made for Depocon, it is recommended as a precaution that Depocon should not be used where there is a history of or existing thromboembolic processes (e.g. stroke, myocardial infarction) or in sickle-cell anaemia.

In earlier years, when estrane derivatives were being used at very high dosage for the maintenance of pregnancy, individual cases of virilisation of the external sex characteristics of female neonates were also described following administration of preparation containing norethisterone, this being associated with the androgenic residual effect of these substances.

As with other steroids it is therefore theoretically possible that the degradation of bilirubin in neonates is impaired particularly during the first week of life. If the new-born is suffering from severe or persistent jaundice which required medical treatment, breast-feeding must be interrupted for the time of treatment.

A reduction of glucose tolerance has been observed in some women using progestogens. Consequently diabetics and women with a tendency to diabetes should be carefully supervised. In cases of diabetes, it may be necessary to reassess the required doses of antidiabetics or insulins.

INTERACTION WITH OTHER MEDICAMENTS:

The doctor should be informed if other medicines are being taken regularly (e.g. barbiturates, phenylbutazone, hydantoins, rifampin), since there

may impair the action of Depocon.

USE IN PREGNANCY & LACTATION:

Use in Pregnancy: Since it cannot be stated unequivocally that such a situation will not occur under Depocon, during pregnancy – and particularly in the sensitive phase after the first month of pregnancy – is contraindicated.

It must be added, however, that no such effect has been observed in the few pregnancies which have so far, occurred during the wearing-off of the Depocon effect.

Use in Lactation: Depocon has no inhibiting influence on lactation. Up to now, no unfavourable effects on breast-fed babies have been observed. However, small quantities of the steroid may be eliminated with the milk.

SIDE EFFECTS:

Cycle disturbances in the form of spotting, breakthrough bleeding and amenorrhoea of short duration. These do not generally require treatment. With persistent bleeding, however, it may be expedient to administer progestogen-estrogen tablets for 10 days, which cause a withdrawal bleeding about 1-4 days after the last tablet has been taken.

If the use of Depocon has been interrupted because of the absence of bleeding, diagnostic measures are called for to clarify the cause. If pregnancy can be ruled out and the amenorrhoea still persists, special treatment is required in younger women in particular. The subjective symptoms reported consist of isolated cases of headaches, dizziness, and transient nausea. Marked weight gains are rare.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Presentation of a single use injectable and administration by a physician minimize the risk of overdose. There have been no reports of serious deleterious effects from overdose. There are no antidotes and further treatment should be symptomatic.

INCOMPATIBILITIES (FOR INJECTIONS ONLY):

In the absence of compatibility studies, this medicinal product should not be mixed with other medicinal products.

STORAGE CONDITIONS:

Store below 30°C. Protect from light. KEEP OUT OF REACH OF CHILDREN. *JAUHKAN DARIPADA KANAK-KANAK.*

SHELF LIFE:

Please refer to outer package.

PACK SIZE:

Box of 10 ampoules x 1 ml, 30 ampoules x 1 ml, 50 ampoules x 1 ml and 100 ampoules x 1 ml.

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD

Lot 2599, Jalan Seruling 59 Kawasan 3,

Taman Klang Jaya, 41200 Klang, Selangor, MALAYSIA.